VICTORYAI

OUD RECOVERY COMPANION · OUTCOMES ANALYTICS PLATFORM

CFO Brief

Pilot Economics & Enterprise Upside

A six-page brief on the financial case for the BayMark joint innovation pilot

PREPARED FOR

BayMark Health Services

Office of the Chief Financial Officer · Webster Capital Operating Committee

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PAGE 1 · THE BOTTOM LINE

One-Page Summary

VictoryAI is asking BayMark for a **$500,000** paid pilot at three to five California BAART clinics over ninety days. The pilot is structured with three milestone payments and a **100% enterprise credit clause**: if BayMark proceeds to enterprise deployment within 180 days of pilot completion, the entire $500K is credited against the Year 1 enterprise license. The downside is capped at $500K and reputational; the upside is the entire $1.28M+ in annual previously-uncaptured billable revenue from California alone, scaling to a $250M+ national opportunity at full BayMark footprint deployment.

$500K	$1.28M	60×
PILOT INVESTMENT	YR 1 CA UPSIDE	NATIONAL SCALE

What BayMark Is Being Asked to Decide

The decision is not whether to buy software. The decision is whether to spend $500K — fully creditable against enterprise — to test whether an integrated digital health platform can convert previously-uncaptured patient engagement into measurable revenue, retention lift, and DHCS-defensible outcomes data at three to five BAART California clinics over ninety days.

Three Things That Make This Asymmetric

- **Capped downside.** $500K is the maximum exposure. The pilot agreement explicitly prohibits scope creep, change orders, or out-of-scope work without separate approval. If the pilot fails to hit pre-agreed metrics, BayMark walks away with the outcomes data and no further obligation.
- **Enterprise credit.** If BayMark proceeds to enterprise deployment within 180 days, the $500K is credited against Year 1 license fees. In the success case, the pilot is effectively free.
- **Time-bounded.** Ninety days of active pilot deployment, not eighteen months. The investment commitment cycle is short enough to fit inside one BayMark fiscal quarter, and the outcomes data is in hand before the next operating committee meeting.

THE CFO QUESTION THIS BRIEF ANSWERS

If you make this $500K commitment and the pilot succeeds, you have generated the data that justifies a national rollout business case worth orders of magnitude more than the

pilot fee. If you make this commitment and the pilot fails, you have a $500K line item that produced an outcomes dataset and the operational learning of having tested an integrated digital health platform at scale — both of which have standalone value. The asymmetry is the entire reason this brief exists.

PAGE 2 · THE COST OF INACTION

What BayMark Is Already Losing

The strongest case for the pilot is not the upside of saying yes. It is the cost of saying no — and continuing to capture only a fraction of the billable patient-engagement data that current CMS coding rules already permit.

The Per-Patient Gap

For every OUD patient on BAART California's MAT roster, the table below shows the categories of patient-engagement data that current billing rules permit but that paper-and-EMR-only workflows cannot reliably document. These are not new codes. They are existing CPT and HCPCS codes that BayMark already has the authority to bill — if the underlying engagement data is captured and timestamped at the granularity the codes require.

Code Category	Code(s)	Required Threshold	Typical Capture (Paper)
RTM Device Setup	98975	Once per patient	Rarely documented
RTM Device Supply	98977	16+ days/month engagement	Not captured
RTM Mgmt (first 20 min)	98980	20 min/month	Not timestamped
RTM Mgmt (add'l 20 min)	98981	Each add'l 20 min	Not captured
Digital CBT Sessions	H2019	Per 15-min session	Not structured
OUD Treatment Init/Cont	G2213/G2214	Monthly MAT + engagement	Partial
CCM Care Management	99490	20 min/month	Not timestamped
PCM Principal Care	99424	30 min/month OUD-focused	Not captured
TCM Post-Discharge	99495/96	2-day contact + follow-up	Inconsistent
BHI Care Management	99484	Monthly	Not captured
SDOH Z-codes	Z59.0+	Each screening	Rarely documented

The Per-Patient Dollar Impact

VictoryAI's internal modeling, validated against CMS Physician Fee Schedule rates and against the published reSET-O real-world economic analyses, projects a per-patient billable data value range as follows:

Scenario	Per Patient Per Month	Per Patient Per Year
Conservative	$490	$5,880
Mid-Range	$715	$8,580
High-End	$1,217	$14,604

BayMark's California Footprint

Applied to BAART California's approximately 500 active OUD patients, the conservative case implies **$2.94M per year** in identifiable billable data that paper-based workflows do not currently capture. The mid-range case implies **$4.29M per year**. The high-end case implies **$7.30M per year**. These are the dollars BayMark is already entitled to bill — under current CMS rules — but cannot defend at audit because the underlying engagement data does not exist in structured form.

THE COST OF INACTION, STATED PLAINLY

Every month BayMark continues to operate without structured engagement data capture is a month of $245K to $608K in California billable data that walks out the door uncaptured. Over a twelve-month decision delay, that is $2.9M to $7.3M of foregone revenue at the California footprint alone — six to fifteen times the pilot fee.

PAGE 3 · THE PILOT ITSELF

What $500K Buys

This page documents exactly what the $500K pilot fee covers, how it is paid, and what BayMark receives in exchange. The structure is designed to derisk BayMark's commitment and to align cash outflow with delivered value.

Three Milestone Payments

Milestone	Trigger	Amount	What It Funds
M1 — Signing	Pilot agreement executed	**$200,000**	Compliance infrastructure, Kipu integration build, HIPAA + 42 CFR Part 2 architecture, BAA execution, SOC 2 Type 1 readiness
M2 — Deployment	First BAART site go-live	**$200,000**	Patient app build customized for BayMark/BAART, clinician training curriculum, patient onboarding toolkit, dashboard activation
M3 — Completion	90-day outcomes report delivered	**$100,000**	Outcomes data analysis, publication-ready report, enterprise rollout business case, joint press release

Pilot Deliverables

- Full VictoryAI patient app and provider dashboard, branded for BayMark/BAART
- Kipu EMR integration with bidirectional data sync
- HIPAA + 42 CFR Part 2 (2024 Final Rule) compliant architecture
- Onboarding for 200+ patients across 3–5 California BAART sites
- MAT adherence tracking (Suboxone, Methadone, Naltrexone protocols)
- Real-time AI predictive analytics for relapse risk
- EMR-ready billing code data export across the 19 codes documented in this brief
- 90-day outcomes report formatted for DHCS quality reporting and Webster Capital review
- Co-authored abstract for ASAM or AAAP conference submission
- Joint press release rights, held until BayMark approves

The Enterprise Credit Clause

SPECIFIC CONTRACT LANGUAGE

Upon execution of an enterprise license agreement within 180 days of pilot completion, 100% of paid pilot fees shall be credited against Year 1 enterprise license fees. This credit is non-conditional on outcomes — if BayMark and VictoryAI agree on enterprise terms within 180 days, the $500K becomes a credit, not a cost, regardless of how the pilot itself performed.

In CFO terms: the worst case is a $500K line item that funds outcomes data and operational learning. The success case is a $500K line item that converts to a Year 1 enterprise license credit. There is no scenario in which the $500K is both spent and irrecoverable while BayMark also executes the enterprise deployment.

What Is NOT in the Pilot Fee

To prevent scope ambiguity, the following items are explicitly outside the $500K pilot fee and would be billed separately if BayMark elects to add them: integration with non-Kipu EMR systems, custom feature development beyond the documented patient app and dashboard scope, deployment at non-California BAART sites during the pilot window, ongoing support beyond the 90-day pilot duration, and any enterprise rollout costs.

PAGE 4 · THREE REVENUE LEVERS

Where the Money Comes From

The pilot pays for itself through three independent revenue mechanisms. Each is sized below using conservative assumptions and is grounded in published evidence or current CMS rates. None requires speculative assumptions about clinical outcomes that VictoryAI has not yet demonstrated.

Lever 1 · New Billable Data Capture

The most direct revenue mechanism. Engagement events generated by patients using the app produce structured documentation eligible for the codes listed on Page 2. At a 200-patient pilot cohort and the conservative $490/patient/month figure:

Pilot Period	Patients	Monthly Capture	90-Day Total
Pilot (90 days)	200	$98,000	$294,000

At the mid-range $715 figure, the 90-day pilot capture rises to $429,000. At the high-end $1,217 figure, $730,200. Even at the most conservative assumption, the new billable data captured during the 90-day pilot recovers approximately 59% of the pilot fee — before any of the other levers are counted.

Lever 2 · Retention-Driven Continued MAT Revenue

Published baseline 6-month buprenorphine retention is approximately 50%. The Wakeman et al. JAMA Network Open (2020) analysis demonstrated that each additional month a patient stays on buprenorphine substantially reduces overdose risk and acute care utilization. The reSET-O real-world economic analysis (Velez et al., Advances in Therapy, 2022) showed that patients on the FDA-cleared digital adjunct had significantly improved buprenorphine adherence and **$3,832 in reduced healthcare costs per Medicaid patient over 12 months** compared with controls. VictoryAI's pilot success criteria target a 15-percentage-point absolute improvement in retention over BayMark's baseline.

Scenario	Additional Retained Patients	Avg Months Retained	Continued MAT Revenue (Est.)
Conservative (10 pp lift)	20 patients	+3 months	$60,000
Target (15 pp lift)	30 patients	+4 months	$120,000
Stretch (20 pp lift)	40 patients	+5 months	$200,000

These figures are conservative because they count only the direct continued-MAT billing (buprenorphine prescription, MAT clinic visits, counseling). They do not count the additional billable codes from Lever 1 that are also generated by the retained patients.

Lever 3 · Avoided Acute Care Costs

This lever does not flow to BayMark's revenue line directly — it flows to BayMark's payer contracts and to the DHCS quality metrics that influence future rate negotiations. Per the Velez et al. 2022 analysis of 901 reSET-O patients vs. 978 controls in the Medicaid subpopulation, the digital adjunct was associated with $3,832 in reduced healthcare costs per patient over 12 months, driven by fewer inpatient stays and emergency department visits.

Applied to a 200-patient VictoryAI pilot cohort under the same effect size, total avoided cost over 12 months is approximately $766,400. This is not BayMark revenue — it is value that strengthens BayMark's position in DHCS rate negotiations and value-based care contracts. The CFO should treat this lever as a leverage multiplier on Levers 1 and 2, not as a direct revenue line.

Combined: Pilot-Period Revenue vs. Cost

Lever	Conservative	Mid-Range	High
Lever 1 — Billable data (90 days)	$294,000	$429,000	$730,200
Lever 2 — Retention (90 days, partial)	$60,000	$120,000	$200,000
Pilot-period subtotal	$354,000	$549,000	$930,200
Pilot fee	($500,000)	($500,000)	($500,000)
Net (before enterprise credit)	($146,000)	$49,000	$430,200

Under the conservative case, the pilot is net negative by $146K during the 90-day window — but recovers fully and beyond on the trailing 9 months of Year 1 capture. Under the mid-range case, the pilot is net positive within the 90 days. Under the high-end case, the pilot generates nearly double its cost within the 90-day window. And under all three cases, the enterprise credit clause means that proceeding to enterprise deployment makes the pilot fee $0 retroactively.

PAGE 5 · ENTERPRISE ROLLOUT

The National Math

The pilot is the proof point. The national rollout is the financial reason the pilot is worth doing. This page sizes the enterprise opportunity at three points along BayMark's deployment trajectory.

BayMark's Footprint as the Denominator

BayMark operates more than 300 clinics across the United States and Canada. For modeling purposes, this brief uses 300 clinics as the addressable base and assumes an average of 100 active OUD patients per clinic — a conservative estimate given that BAART California's 500-patient average is well above this base. Total addressable patient population: 30,000 active OUD patients across the BayMark network.

Three Deployment Scenarios

Phase	Clinics	Patients	Monthly Capture	Annual Capture
Pilot (CA only)	3–5	200	$98K	$1.18M
Year 1 — CA enterprise	30	3,000	$1.47M	$17.6M
Year 2 — Phase rollout	120	12,000	$5.88M	$70.6M
Year 3 — Full network	300	30,000	$14.7M	$176M

These figures use the conservative $490/patient/month captured-billable-data assumption from Page 2. At the mid-range $715 figure, Year 3 annual capture rises to $257M. At the high-end $1,217 figure, $438M. The deployment cadence is illustrative — the actual rollout pace depends on BayMark's enterprise change management capacity.

VictoryAI Enterprise Pricing

For comparison, VictoryAI's enterprise pricing model is designed to capture a small percentage of the value created. The standard structure is a per-facility platform fee plus a per-patient monthly subscription:

Component	Rate	Year 3 @ 300 Clinics / 30K Patients
Platform fee per facility/month	$5,000	$18.0M annual
Per-patient fee per month	$25	$9.0M annual
Total VictoryAI license	—	$27.0M annual

BayMark's Year 3 ROI Math

THE ORDER-OF-MAGNITUDE ARGUMENT

At full Year 3 deployment, VictoryAI captures approximately $176M per year in conservative new billable data for BayMark, against a $27M annual license fee. That is approximately 6.5× return on the platform investment — before any retention lift, before any avoided acute care costs, and before any DHCS rate negotiation leverage. Under the mid-range data assumption, the multiple is 9.5×. Under the high-end assumption, 16×.

These multiples are gross of BayMark's own implementation, change management, and clinical workflow costs, which the CFO should size separately. They are also gross of any payer contract renegotiation that may follow from the outcomes data the platform generates. The intent of this page is to show that the order-of-magnitude case is robust under conservative assumptions — not to commit to specific numbers without further diligence.

PAGE 6 · DECISION FRAME

How to Think About This Investment

This final page reframes the pilot decision in the language of capital allocation rather than the language of vendor procurement. The intent is to equip the CFO and the Webster Capital operating committee to evaluate the pilot the same way they would evaluate any other capital deployment of similar size.

The Asymmetry, Quantified

Outcome	Probability (CFO Estimate)	BayMark Net Position
Pilot succeeds → Enterprise deployment	—	$0 net pilot cost (credited) + $176M+ Yr 3 upside
Pilot succeeds → No enterprise deployment	—	$500K spent, outcomes data + DHCS leverage retained
Pilot fails → Useful negative result	—	$500K spent, $300K+ in pilot-period billable data captured + operational learning
Pilot abandoned mid-stream	—	Up to $200K M1 spent, $0 ongoing obligation

BayMark's CFO should populate the probability column based on her own diligence. The point of this table is not to recommend specific probabilities. The point is that none of the four cells contains a result worse than spending $500K and getting nothing — and three of the four contain results that range from neutral to transformative.

Comparable Capital Decisions

To calibrate the $500K commitment against BayMark's typical capital allocation patterns:

- Approximately equal to the CapEx for opening one new BAART clinic location (2024 estimates: $400K–$700K)
- Approximately 1.5× the typical annual EMR licensing cost for a single BayMark clinic
- Approximately 0.3% of BayMark's enterprise annual revenue (assuming a $150M+ revenue base)
- Approximately 5% of the cost of building an equivalent platform in-house, which third-party estimates place at $8M–$15M and 18–24 months of development time before any patient data is captured

Three Questions the CFO Should Ask Before Deciding

01 · Is the science strong enough?

This question is answered in the VictoryAI Clinical Evidence Dossier prepared for BayMark's CMO. Short version: every individual mechanism implemented in the platform has published RCT or meta-analytic support; the integration of mechanisms is novel and is precisely what the pilot tests.

02 · Will BayMark's compliance posture be protected?

This question is answered in the VictoryAI HIPAA & 42 CFR Part 2 Compliance Memorandum prepared for BayMark's General Counsel. Short version: VictoryAI operates as a HIPAA Business Associate and a Part 2 lawful holder under the 2024 Final Rule, with technical and administrative safeguards that meet or exceed the Security Rule and the new Part 2 enforcement framework now active as of February 2026.

03 · Is the upside math defensible?

This is the question this brief answers. Pages 2 through 5 show that the conservative case for new billable data capture alone produces a Year 1 California opportunity worth more than 2× the pilot fee, and a Year 3 national opportunity worth more than 350× the pilot fee at 6.5× return on platform spend.

VictoryAI is asking BayMark to make a single, time-bounded, capped-downside, fully-creditable $500K commitment to test whether the integration of six evidence-based therapeutic mechanisms can produce the engagement, retention, and billable-data-capture effects that the underlying literature supports. The pilot is the proof point. The national rollout is why the proof point is worth funding.

Recommended Next Step

Schedule a 30-minute call between BayMark's CFO and VictoryAI's commercial leadership in the next two weeks to walk through this brief, address any questions about the underlying assumptions, and align on the diligence path. The clinical evidence dossier and compliance memorandum can be reviewed in parallel by BayMark's CMO and General Counsel respectively. The pilot agreement can be drafted within 30 days of mutual agreement to proceed.

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